MEDIBANK HEALTHCARE INTELLIGENCE BRIEFING

Date: October 2026

Classification: For Healthcare Executive Review

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1. EXECUTIVE SUMMARY

Australia's health system continues to experience significant expenditure growth, with total spending reaching $290.7 billion in 2024–25 (10.5% of GDP). Hospital spending is the dominant cost driver, growing 4.0% in real terms, while private health insurance provider spending grew modestly at 2.1%. Disease expenditure analysis reveals cancer as the highest-cost disease group ($21.7 billion), chronic conditions accounting for 55% of all disease spending, and significant health equity gaps between advantaged and disadvantaged populations. New data on polypharmacy, diabetes prevalence, mental health in primary care, substance use trends, and e-mobility injuries present both risks and opportunities for Medibank's product design, claims management, and member health programs.

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2. KEY PRIORITISED INSIGHTS

2.1 Hospital Spending Acceleration — Particularly Public Hospitals

Priority: HIGH

Finding: Hospital spending reached $123.4 billion in 2024–25, a 4.0% real increase. Public hospital spending grew 5.2% in real terms to $99.9 billion, above the decade average of 4.6%. Private hospital spending was $23.5 billion, with non-government spending on private hospitals *decreasing* 1.6% in real terms and private hospital separations declining slightly by 0.3%.

Key statistics:

• PHI providers spent $11.1 billion on private hospitals in 2024–25

• PHI provider spending per person covered was $1,779, a real decrease of $8 (0.4%) from 2023–24

• State/territory government spending on public hospitals rose 6.4%

• Public hospital separations increased 2.3%; private hospital separations decreased 0.3%

Why it matters to Medibank: The divergence between growing public hospital activity and flat/declining private hospital utilisation signals potential member migration toward the public system. The slight decline in PHI spending per person covered may reflect competitive pressure on premiums, but also raises questions about whether members are accessing fewer private hospital services. Medibank should monitor whether this reflects affordability constraints, changing member preferences, or shifts in elective surgery patterns.

Source: Document 1 – AIHW Health expenditure Australia 2024–25

2.2 Cancer Spending Dominance and Rapid Growth

Priority: HIGH

Finding: Cancer had the highest disease spending at $21.7 billion in 2024–25, more than doubling from $9.1 billion in 2013–14. Cancer has been the top-ranked disease group since 2017–18. The top five cancers by spending were non-melanoma skin cancer ($2.12B), breast cancer ($2.09B), lung cancer ($2.04B), bowel cancer ($1.9B), and prostate cancer ($1.8B). PBS spending on cancer has been more than double the next highest disease group for the past five years.

Key statistics:

• Cancer spending grew 73% in real terms since 2013–14

• Cancer was also the highest burden of disease group (16.4% of total DALYs in 2024)

• $508.7 million spent on national cancer screening programs in 2024–25

• National Lung Cancer Screening Program implemented July 2025

Why it matters to Medibank: Cancer treatment costs are a major claims driver. The rapid growth in PBS cancer spending and the introduction of new screening programs (lung cancer) will increase early detection and potentially shift treatment patterns. Medibank should anticipate rising claims from earlier-stage cancer diagnoses, new high-cost therapies (particularly immunotherapies and targeted treatments), and consider how screening program participation among members can be incentivised to improve outcomes and manage long-term costs.

Source: Document 2 – AIHW Health system spending on disease and injury in Australia 2024–25

2.3 Chronic Conditions Drive Majority of Disease Spending Growth

Priority: HIGH

Finding: Spending on chronic conditions accounted for $107.6 billion (55%) of all disease spending in 2024–25. Of the $94.1 billion growth in disease spending since 2013–14, 60% ($56.8 billion) was attributable to chronic conditions. The top conditions by spending were injuries from falls ($5.7B), osteoarthritis ($5.2B), back pain and problems ($4.3B), coronary heart disease ($4.0B), and lower respiratory infections ($3.8B).

Key statistics:

• Population growth contributed 44.3% of spending growth; expenditure per service 33.6%; population ageing 16.0%

• Musculoskeletal disorders had the highest private hospital spending ($5.8B)

• Spending peaked in the 75–79 age group for both sexes in 2024–25, shifting from 70–74 in prior years

Why it matters to Medibank: Chronic disease management is central to Medibank's value proposition. The upward shift in peak spending age (now 75–79) reflects population ageing and has direct implications for premium setting and reserve adequacy. Musculoskeletal disorders dominating private hospital spending means joint replacements and spinal procedures remain key claims categories. Investment in chronic disease management programs, particularly for osteoarthritis and cardiovascular disease, could yield significant returns.

Source: Documents 2 and 3 – AIHW Health system spending on disease and injury; Growth decomposition

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2.4 Diabetes Prevalence, Gestational Diabetes Surge, and GLP-1 Prescribing Explosion

Priority: HIGH

Finding: About 1.3 million Australians (1 in 15) were living with diabetes in 2022–24. Gestational diabetes incidence doubled between 2012–13 and 2023–24, now affecting almost 1 in 5 women giving birth. GLP-1 analogue prescriptions surged 87% from 1.7 million to 3.2 million between 2021–22 and 2024–25. Diabetes accounted for $4.4 billion in health expenditure (2.4% of total) and 11% of all hospitalisations in 2023–24.

Key statistics:

• Diabetes prevalence 3.5× higher in most disadvantaged vs least disadvantaged areas

• First Nations adults 3.2× as likely to have diabetes

• Semaglutide (Ozempic) was the 7th most costly PBS medicine in 2024–25

• 88% of adults with diabetes were overweight or obese

Why it matters to Medibank: The GLP-1 prescribing surge has major pharmaceutical cost implications. The gestational diabetes doubling affects maternity claims and creates a pipeline of women at elevated risk for future type 2 diabetes. Medibank should model the cost trajectory of GLP-1 therapies, consider targeted diabetes prevention programs (particularly for gestational diabetes follow-up), and assess whether chronic disease management programs adequately address the socioeconomic gradient in diabetes burden.

Source: Document 7 – AIHW Diabetes: Australian facts

2.5 Health Spending Exceeds GDP Growth; Health-to-GDP Ratio Rising

Priority: HIGH

Finding: Health spending represented 10.5% of GDP in 2024–25, up 0.4 percentage points from 2023–24. Health prices grew 3.89%, exceeding general inflation of 3.3%, producing "excess health inflation"

of 0.57%. Government funded 70.0% of total health expenditure. The Australian Government's share of total health expenditure was at its lowest over the decade (39.2%), while state/territory share was at its highest (30.8%).

Key statistics:

• Per person health spending: $10,614 (up 1.8% in real terms)

• Non-government spending: $87.1 billion (30% of total)

• Individual out-of-pocket spending: $46.4 billion ($1,693 per person, up 3.6% in real terms)

• PHI premium rebate: $7.2 billion (up 1.0% in real terms)

Why it matters to Medibank: Excess health inflation directly pressures premium affordability. The rising health-to-GDP ratio and shifting government funding shares signal potential policy changes. The Australian Government's declining share may lead to pressure on private insurers to absorb more costs. Rising out-of-pocket costs ($1,693 per person) may increase member sensitivity to gap payments and drive demand for more comprehensive cover or, conversely, downgrading of policies.

Source: Document 1 – AIHW Health expenditure Australia 2024–25

2.6 Polypharmacy Affects 1.9 Million Australians; 40% of Over-75s

Priority: MEDIUM

Finding: In 2025, 7% of Australians (1.9 million people) were regularly dispensed 5+ medicines simultaneously. Among people aged 75+, 40% experienced polypharmacy. Around 1 in 9 people experiencing polypharmacy (219,000) were on 10+ medicines (hyperpolypharmacy). Mental health medicines dominated in younger polypharmacy patients; chronic disease medicines in older adults. Antidepressants were common across most age groups.

Key statistics:

• 8 million people (29% of population) were taking 1–4 regular medicines

• Polypharmacy rates similar between males and females overall

• Males aged 65–74 had higher polypharmacy rates (22%) than females (18%)

Why it matters to Medibank: Polypharmacy increases the risk of adverse drug reactions, hospitalisations, and medication non-adherence — all of which drive claims costs. Medibank could leverage this data to support medication review programs for high-risk members, particularly those aged 75+ on multiple medicines. Integration with pharmacy benefit management and chronic disease programs could reduce preventable hospitalisations.

Source: Document 4 – AIHW Polypharmacy in Australia in 2025

2.7 Mental Health: Significant Physical Health Comorbidity and Higher GP Utilisation

Priority: MEDIUM

Finding: A proof-of-concept study using WA general practice data found that up to 4 in 10 people with mental health conditions have co-existing physical health conditions, and up to 9 in 10 have recorded biomedical risk factors or health behaviours. People with mental health conditions attend GPs more than twice as frequently as those without (median 6–10 visits vs 3). People with schizophrenia/affective psychosis had nearly double the rate of type 2 diabetes (13%) compared to those without mental health conditions (6.7%).

Key statistics:

• Mental health conditions ranked 5th for disease spending but 2nd for disease burden

• When community/public health spending is included, mental health moves to 3rd highest spending group

• Obesity rates nearly double for people with schizophrenia/psychosis (38%) vs those without (21%)

Why it matters to Medibank: The physical health burden of mental health conditions represents a significant, often under-addressed cost driver. Medibank's mental health programs should integrate physical health screening and management. The high GP utilisation among mental health patients suggests opportunities for coordinated care models that could improve outcomes and reduce downstream hospital costs.

Source: Documents 2 and 8 – AIHW Disease spending; General practice care for people with a mental health condition

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2.8 Spending Disparities by Remoteness and Socioeconomic Status

Priority: MEDIUM

Finding: Cost per service was consistently higher for people in Remote/Very Remote areas and most disadvantaged areas across all disease groups from 2013–14 to 2024–25. Hospitalisation expenditure per person for First Nations people was 1.6× that of non-Indigenous people ($5,683 vs $3,508). For mental health conditions, the ratio was 2.9×. First Nations expenditure per person was *lower* for cancer ($290 vs $372), musculoskeletal disorders ($332 vs $379), and hearing/vision disorders ($69 vs $106).

Key statistics:

• Cancer cost per service: Very Remote $895 vs Major Cities $633

• Cardiovascular cost per service: Very Remote $483 vs Major Cities $204

• Potentially preventable hospitalisation expenditure for chronic conditions: First Nations $345 vs non-Indigenous $128 per person

Why it matters to Medibank: These disparities highlight both equity concerns and potential market opportunities. Medibank's regional and rural member base may face higher per-service costs. The lower cancer and musculoskeletal spending for First Nations people may indicate access barriers rather than lower

need, suggesting unmet demand. Telehealth and digital health investments could help address access gaps for remote members.

Source: Document 2 – AIHW Health system spending on disease and injury in Australia 2024–25

2.9 Daily Tobacco Smoking at Record Low; Vaping Stabilises; Illicit Tobacco Doubles

Priority: MEDIUM

Finding: Daily tobacco smoking fell to a record low of 5.6% in 2025 (down from 8.3% in 2022–23). Daily vaping stabilised at 3.6%. However, illicit tobacco use among current smokers doubled from 17% to 34% between 2022–23 and 2025. Australia had the second-lowest daily smoking rate among OECD countries. Methylamphetamine consumption increased 23% nationally; cocaine increased 17%.

Key statistics:

• Estimated social cost of tobacco: $155.4 billion (2020–21)

• Estimated social cost of alcohol: $72.9 billion (2020–21)

• GHB hospitalisations surged from 569 (2015–16) to 3,674 (2023–24)

• Methamphetamine hospitalisations peaked at 55.9 per 100,000 in 2023–24

Why it matters to Medibank: Declining smoking rates should gradually reduce tobacco-related claims over time. However, the rise in illicit tobacco, vaping, and stimulant use introduces new health risks. GHB-related hospitalisations have increased dramatically and may affect younger demographics. Medibank should monitor emerging substance-related hospitalisation trends for claims forecasting and consider how prevention programs can address these shifting patterns.

Source: Document 5 – AIHW Alcohol, tobacco & other drugs in Australia

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2.10 E-Scooter and E-Bike Injuries Surging, Especially Among Young Males

Priority: WATCHLIST

Finding: Hospitalisations for falls involving powered scooters tripled since 2019–20, reaching 15 per 100,000 in 2024–25. Boys aged 12–15 had the highest rate at 54 per 100,000 — a 15-fold increase since 2019–20. Fractures accounted for 58% of powered scooter hospitalisations. In-hospital deaths increased from 7 (2015–16) to 20 (2024–25). Rates were highest in Queensland and Western Australia.

Key statistics:

• 4,089 hospitalisations for falls involving powered scooters in 2024–25

• 2,297 hospitalisations for motor-scooter/moped/motorised bicycle riders

• 47% of powered scooter falls occurred on streets/highways (up from 20% in 2015–16)

• New ICD-10-AM codes from July 2026 will improve data specificity

Why it matters to Medibank: E-mobility injuries represent an emerging claims category, particularly for orthopaedic and trauma services. The concentration among young males (12–15 years) has implications for family policies. Medibank should monitor this trend for product pricing, particularly in states with more permissive e-scooter regulations, and consider whether injury prevention messaging to members is warranted.

Source: Document 9 – AIHW E-scooter and e-bike injuries and deaths in Australia

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3. EMERGING WATCHLIST

Item	Signal	Timeframe	Source
National Primary Health Care Data Collection	AIHW developing national GP data collection with PHNs; will transform primary care visibility	2026–2028	Document 8
GLP-1 analogue cost trajectory	87% growth in prescriptions; Semaglutide 7th most costly PBS medicine; potential expansion to obesity indications	12–24 months	Document 7
Spending peak shifting to 75–79 age group	First time peak disease spending shifted from 70–74 to 75–79 for both sexes	Ongoing	Document 2
Non-government primary care spending now largest PHC share	First time non-government sources exceeded Australian Government share of primary health care spending	Structural shift	Document 1
NDIS health spending exclusion	$5.5 billion in NDIS health-related spending not yet included in national health accounts	Policy risk	Document 1
National Lung Cancer Screening Program	Implemented July 2025; will increase early-stage lung cancer diagnoses and treatment costs	12–36 months	Document 2
Rising methylamphetamine and GHB hospitalisations	Methamphetamine hospitalisations at record high; GHB hospitalisations up 6.5× since 2015–16	Ongoing	Documents 5, 6
ICD-10-AM e-mobility codes (July 2026) and ICD-11 transition (2028–2031)	Will improve injury data specificity; ICD-11 implementation 2028/29 for mortality, 2031 for hospitalisations	2026–2031	Document 9

This briefing is based solely on the supplied AIHW source documents. All statistics and findings are attributed to the relevant source document as indicated.